

# The 2026 Healthcare and DME Public-Sector Opportunity Report

A practical guide to VA procurement, Medicare supplier signals, funded providers, recipients, channel partners, and source-backed next actions.

|                                                                                    |                                                                              |                                                                                   |
|------------------------------------------------------------------------------------|------------------------------------------------------------------------------|-----------------------------------------------------------------------------------|
| <div>14 signals</div> <div>Sourced in a real healthcare and DME company scan</div> | <div>5 agencies</div> <div>Distinct federal buyers or funders surfaced</div> | <div>\$95.9M</div> <div>Largest historical award record in the evidence set</div> |
|------------------------------------------------------------------------------------|------------------------------------------------------------------------------|-----------------------------------------------------------------------------------|

Built for founders and revenue teams that need credible targets, clear revenue motions, contact paths, and next actions - not a generic list of grants or contracts.

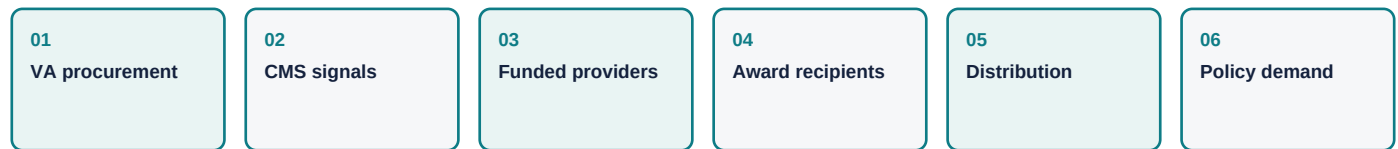
## Opportunity Scanner by Opportunity Systems

Research current as of July 27, 2026. Always recheck time-sensitive official sources before acting.

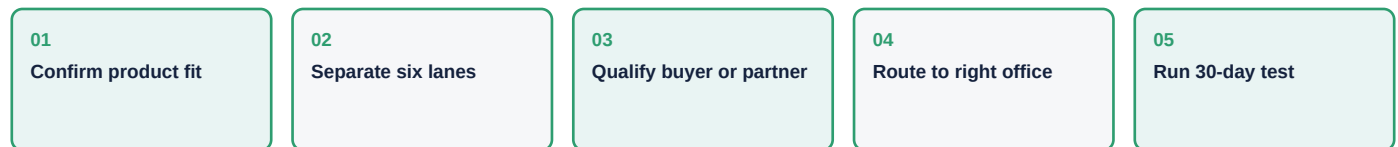
# The opportunity landscape

Use these lanes to separate records that imply different buyers, partners, timing, and next actions.

## 6 evidence lanes to investigate



## From signal to an owned revenue action



These diagrams are operating maps, not a promise that every lane is open. The report explains the evidence, constraints, and qualification checks behind each route.

Updated July 27, 2026. Time-sensitive Medicare enrollment and competitive-bidding information must be rechecked before publication and on every update.

## Offer and audience

This report is for healthcare suppliers, DMEPOS companies, medical-supply manufacturers and distributors, rehabilitation and mobility businesses, healthcare technology companies, and revenue teams evaluating public-sector growth.

Conversion promise: Understand the distinct public-sector opportunity paths in healthcare and use a repeatable method to qualify them. The report does not determine Medicare eligibility, accreditation, coding, reimbursement, contracting eligibility, clinical compliance, or revenue potential.

## Executive brief

Healthcare public-sector opportunity is not one market. It includes agency procurement, Federal Supply Schedule routes, Medicare supplier and reimbursement requirements, publicly funded health centers and providers, grant recipients, distributors, prime vendors, and policy signals. Each lane uses different evidence and demands a different action.

National health spending provides context for the scale and policy importance of healthcare, but it is not the addressable market for a DME or medical-supply company. CMS projects national health expenditure growth and publishes Medicare data, fee schedules, supplier guidance, and policy changes. VA publishes medical equipment and supply schedule information. HRSA publishes current and historical grant and health-center data. SAM.gov and USAspending provide procurement and federal award evidence.

The highest-value work is separating these lanes before outreach. A reimbursement rule may require Monitor Policy or supplier-readiness work. A VA schedule may support a contracting pathway. A HRSA recipient may support funded-buyer or partner research. A prior award may reveal an incumbent, distributor, or agency pattern without proving current demand.

The correct output is a focused action table in which every row identifies offer fit, official evidence and period, target, route, constraint, and next validation task. A compliance requirement can be strategically important without being a sales lead.

## What the real scan data shows

An anonymized scan of a compression-garment and recovery-products brand surfaced 14 sourced signals across five federal agencies and two source systems. These are historical award records, not open opportunities. They show that the DME-adjacent market has several different buyer and partner routes:

| Award year | Public record                                            | Amount       | What it proves                                                                                                                                                                                                     |
|------------|----------------------------------------------------------|--------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 2018       | Peraton Enterprise Solutions, HHS/CMS                    | \$95,922,714 | Medicare claims-processing infrastructure included durable medical equipment. This is enterprise ecosystem evidence, not a direct product order. <a href="#">USAspending record</a>                                |
| 2021       | Valiant Construction LLC, Department of Veterans Affairs | \$17,139,378 | VA funded design-build work for an Omaha physical-therapy and prosthetics clinic. This proves facility investment around the care pathway, not an open medical-supply purchase. <a href="#">USAspending record</a> |
| 2019       | BSN Medical Inc., NASA                                   | \$133,587    | A medical-products supplier received a NASA award, showing that relevant direct buyers can appear outside the agencies a seller would normally target. <a href="#">USAspending record</a>                          |

The operating implication is a three-tier buyer map: reimbursement infrastructure, VA care and facility ecosystems, and direct agency supply buying. Each tier needs a different target, contact path, and qualification step.

# State of the market and opportunity signals

## Trend 1: Healthcare spending and public funding remain large, but broad totals require restraint

CMS's current National Health Expenditure fact sheet projects average NHE growth of 5.4 percent from 2025 through 2034, with Medicare spending projected to grow 7.7 percent per year on average. HRSA reports more than 5,500 grant awards annually and reports over \$11.1 billion in FY2025 grants across primary care, health workforce, HIV/AIDS, maternal and child health, rural health, and other programs.

Business implication: Public money affects many healthcare organizations and delivery systems, but those totals combine categories that are not interchangeable and do not measure DME demand.

Recommended action: Use program, recipient, service, product, geography, and time filters to find the narrow records that map to the offer.

## Trend 2: Medicare supplier participation is a readiness and compliance lane, not a generic sales lead

CMS states that DMEPOS suppliers seeking Medicare reimbursement generally must obtain accreditation from a CMS-approved organization, enroll in Medicare, and post a \$50,000 surety bond for each NPI they maintain, subject to stated exemptions and program details.

Business implication: A reimbursement or supplier record may identify market structure, competitors, referring patterns, or compliance work. It does not automatically create a buyer or open opportunity.

Recommended action: Route Medicare signals to supplier-readiness, reimbursement, compliance, product-category, or provider-market research before deciding whether outreach is appropriate.

## Trend 3: A temporary 2026 enrollment moratorium changes the direct supplier path

CMS reports that on February 27, 2026 it implemented a temporary nationwide moratorium on initial Medicare enrollment for specified DMEPOS medical supply company categories. CMS states the moratorium lasts six months and may be extended in six-month increments.

Business implication: A company affected by the moratorium should not interpret supplier-enrollment content as an immediately available path. The status is time-sensitive and does not eliminate separate procurement, distribution, manufacturer, technology, service, funded-provider, or partner opportunities.

Recommended action: Verify the current CMS moratorium page and definitions before taking enrollment action. Consult the appropriate enrollment contractor or qualified advisor for company-specific questions.

## Trend 4: VA procurement includes category-specific schedule routes

VA's Schedule 65 II A covers medical equipment and supplies across categories such as surgical dressings, instruments, hospital equipment, clothing, ophthalmic equipment, kits, and medical equipment maintenance. VA identifies SAM.gov as the official source for government contract opportunities while publishing schedule solicitation materials and ordering information.

Business implication: Product category, Federal Supply Code, NAICS, schedule requirements, and ordering route matter. A company may also participate through a distributor, schedule holder, prime, or subcontracting relationship.

Recommended action: Map products to the applicable VA schedule and codes, inspect active notices separately, and compare direct schedule, agency procurement, distributor, and partner motions.

## Trend 5: Funded providers and recipients create a distinct buyer and partner lane

HRSA publishes active and awarded grants, health-center awardee data, and service-delivery sites. HRSA notes that FY2026 award data is preliminary and current-year-to-date, and defines an active grant by project-period end date rather than proof of new current-year funding.

Business implication: A funded health center or recipient may be a buyer, partner, implementation site, or research target, but the funding purpose and active project scope must fit the offer.

Recommended action: Verify program purpose, project period, recipient role, and likely purchasing authority before creating outreach.

## Trend 6: Round 2028 competitive bidding is a readiness signal, not yet a contract award

CMS states that the program is in a temporary gap after Round 2021 contracts expired on December 31, 2023. Round 2028 covers Class II continuous glucose monitors and insulin pumps, urological supplies, ostomy supplies, hydrophilic urinary catheters, and off-the-shelf back, knee, and upper-extremity braces. Published registration and bidding dates remain targets; CMS says the round will begin no later than January 1, 2028.

Business implication: Affected suppliers have a readiness lane around enrollment records, licensing, category/location accreditation, and partner strategy. The announcement does not mean registration is open or a contract is available.

Recommended action: Assign an owner to monitor the official CMS and Competitive Bidding Implementation Contractor pages. Inventory category and location accreditation now, but verify actual registration and bid-window dates before representing the opportunity as active.

## Six healthcare opportunity lanes

| Lane                                          | Evidence                                                         | Typical revenue motion                                   | First validation                                            |
|-----------------------------------------------|------------------------------------------------------------------|----------------------------------------------------------|-------------------------------------------------------------|
| VA and agency procurement                     | Schedule pages, SAM.gov notices, award history, vendor portals   | Sell to Agency; Channel / Distributor or Motion          | Product code, notice status, contract vehicle, buyer route  |
| Medicare supplier and reimbursement           | CMS enrollment, accreditation, fee schedule, policy, claims data | Monitor Policy; Research Only; supplier-readiness        | Current rule, eligibility, code, geography, data period     |
| Funded providers and health centers           | HRSA grants, USAspending, recipient and site data                | Sell to Funded Buyer; Partner with Recipient             | Program purpose, project period, purchasing role            |
| Prime, distributor, and manufacturer channels | Award records, VA catalogs, SBA subcontracting, recipient sites  | Sell to Award Recipient; Channel / Distributor or Motion | Existing vehicle, category coverage, partner fit            |
| Direct grants and assistance                  | Grants.gov and agency program notices                            | Direct Apply; Partner with Recipient                     | Eligibility, status, deadline, allowable use                |
| Policy and emerging demand                    | Federal Register, CMS rules, agency guidance                     | Monitor Policy; Research Only                            | Effective date, final versus proposed rule, business impact |

## Healthcare source and action map

Healthcare research becomes safer when each source is assigned a narrow job.

| Source record                    | Verify                                                                 | Contact path                                              | Do not infer                                   |
|----------------------------------|------------------------------------------------------------------------|-----------------------------------------------------------|------------------------------------------------|
| CMS enrollment/accreditation     | Applicable supplier type, NPI, location, category, exemption           | NPE contractor, approved accreditation, qualified advisor | Compliance guidance is a buyer lead            |
| CMS fee schedule/public-use data | Year, geography, HCPCS, population, suppression, payment definition    | Reimbursement lead or payer-policy route                  | Claims or allowed amounts equal current demand |
| CMS competitive bidding          | Final versus target date, covered category/location, enrollment record | CBIC education/help; NPE contract or                      | Preparation guidance is an open bid            |
| VA FSS schedule/catalog          | Exact item, category, holder, terms, purchasing priority               | FSS help, holder, distributor, contracting office         | Schedule coverage is an active order           |

| Source record            | Verify                                               | Contact path                                | Do not infer                              |
|--------------------------|------------------------------------------------------|---------------------------------------------|-------------------------------------------|
| SAM.gov notice/award     | Type, amendment, due date, codes, set-aside, vehicle | Notice instructions and contracting contact | Award or sources sought is a bid          |
| HRSA grant/health center | Purpose, project period, operator, purchasing role   | Recipient procurement or program operations | Grant amount is available for the product |
| Grants.gov opportunity   | Posted status, eligibility, deadline, cost share     | Listed grants contact                       | Topic fit establishes eligibility         |

The table deliberately separates buyer-facing routes from readiness routes. A company can have a high-priority compliance task and no supported outreach target; that result is more useful than forcing the record into a sales sequence.

## The healthcare opportunity workflow

### Step 1: Build a product and readiness profile

Record products, HCPCS or product categories when known, clinical and non-clinical use cases, customers, delivery geography, licenses, certifications, accreditation, reimbursement status, manufacturing or distributor relationships, and implementation capacity. Unknown fields should become validation tasks, not assumptions.

Use one row per offer. Capture exact item and setting; verified HCPCS, FSC, NAICS, or schedule category; commercial role; geography and service model; enrollment, accreditation, licensing, and vehicle readiness; and unresolved evidence gaps. Do not assign an HCPCS code from marketing language alone.

### Step 2: Translate into public-sector lanes

Map each offer to the six lanes above. A mobility product may map to VA patient mobility schedules, rehabilitation programs, DMEPOS policy, funded providers, and distributor partners. A healthcare software product may map to health-center recipients, agency modernization, prime implementers, and program operations without being a DMEPOS supplier.

Select no more than two primary lanes for the first 30 days. Ask what event would create revenue: an agency order, a provider purchase, a partnership, or compliant furnishing and reimbursement. If the event cannot be stated plainly, the lane is exploratory.

### Step 3: Search the source that matches the question

Use VA schedule and procurement pages for category and vehicle research; SAM.gov for active federal notices; USAspending for award and recipient patterns; CMS for reimbursement, supplier, fee schedule, and claims context; HRSA for recipients, active grants, and health-center sites; Grants.gov for open assistance; Federal Register for policy changes.

Preserve each record's native unit and period. Claims, services, suppliers, allowed amounts, obligations, outlays, awards, and sites are not interchangeable and should not be added into an opportunity-size estimate.

### Step 4: Verify status and constraint

Check notice type, deadline, amendment, project period, data year, proposed versus final rule, enrollment restriction, and the definition behind every metric. Recheck the 2026 DMEPOS moratorium immediately before publishing or advising action.

For the moratorium, verify whether the action is an initial enrollment or non-exempt change in majority ownership and whether a listed company type applies. For Round 2028, distinguish preparation guidance from an announced registration or bid window.

### Step 5: Choose target and revenue motion

Name the agency, provider, recipient, prime, distributor, manufacturer, program office, or policy owner. Then select one motion. Do not turn a CMS data record into a sales lead unless a separate buyer path exists.

Separate the **clinical user**, **economic buyer**, **procurement route**, and **compliance owner**. Outreach follows the economic and access roles; discovery incorporates the clinical user.

## Step 6: Build a compliant contact path and next action

Prefer procurement contacts, schedule support, program offices, official vendor registration, recipient leadership, prime or distributor business-development roles, and documented source contacts. The next action should validate category fit, active need, purchasing authority, or partner role.

Match the opening to the lane. Follow notice instructions for agency procurement; give a VA holder or distributor exact category fit and delivery capability; ask a funded provider how related purchasing is handled without implying grant money is available; and route Medicare readiness questions to the official contractor, accreditor, or qualified advisor. Source contacts may be intended for procedural questions, not promotion.

## Healthcare qualification gates

Score fit High, Medium, or Low, then apply these gates before a row reaches workflow:

| Gate           | Pass condition                                                                                          | Downgrade or reject when                                            |
|----------------|---------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------|
| Product gate   | Exact product/service and use case match the record                                                     | Match depends on generic terms such as medical, equipment, or rural |
| Period gate    | Notice, rule, award, project, or dataset period is recorded and useful                                  | Deadline passed, data lag is material, or status is unknown         |
| Authority gate | Buyer, recipient, contract holder, or compliance owner can be named                                     | Only a clinical user or unrelated directory entry is known          |
| Route gate     | A bid, order, billing, distributor, partner, or monitoring path is supported                            | The record shows context but no legitimate action route             |
| Readiness gate | Known enrollment, accreditation, licensing, vehicle, geography, and capacity constraints are manageable | A hard requirement is unmet or not yet understood                   |
| Evidence gate  | Official source supports the plain-language claim and caveat                                            | The interpretation exceeds the source definition                    |

Mark **Pursue**, **Validate**, **Monitor**, or **Reject**. Record why: wrong product/entity, expired, ineligible, geography, vehicle or compliance gap, no purchasing path, duplicate, or stale data. Every monitor row needs a trigger and recheck date.

## Worked illustrative example

Company context: A manufacturer sells rehabilitation supports and related clinical supplies through distributors.

Evidence: VA Schedule 65 II A includes relevant medical and surgical supply categories; an historical federal award shows an incumbent supplier serving a related category; a HRSA-funded provider operates rehabilitation services in the target region.

Interpretation: The schedule is a contracting route, the award is market evidence, and the funded provider is a possible buyer or partner. None proves an active purchase.

Primary motion: Channel / Distributor Motion through a qualified schedule holder or incumbent supplier.

Secondary research: Validate whether the funded provider buys the category and whether a separate local procurement route exists.

Next action: Map exact product codes and schedule requirements, then identify the distributor or contract owner responsible for the category. This is illustrative, not a customer result.

## 30-day action plan

| Period    | Work                                                                      | Deliverable                                     |
|-----------|---------------------------------------------------------------------------|-------------------------------------------------|
| Days 1-5  | Build offer-level readiness rows; verify codes and gaps; choose two lanes | Readiness inventory                             |
| Days 6-10 | Check moratorium, VA route, Round 2028 relevance, and official sources    | Constraint memo and 15-25 authenticated records |

| Period     | Work                                                                                           | Deliverable                             |
|------------|------------------------------------------------------------------------------------------------|-----------------------------------------|
| Days 11-17 | Apply gates and rejection codes; shortlist up to 10 signals                                    | Five rows selected for validation       |
| Days 18-24 | Map user, buyer, access route, compliance owner, and contact path; resolve the largest unknown | Decision-ready action rows              |
| Days 25-30 | Prepare approved outreach or readiness tasks; schedule status rechecks; review lessons         | Owned actions and next-month scan brief |

Track exact offer match, official source, period, target, route, constraint, next action, owner, and recheck date. Commercial outcomes include qualified conversation, partner review, bid/no-bid decision, readiness work, or disqualification.

## Limits and warnings

- This report is not legal, clinical, coding, accreditation, reimbursement, procurement, or eligibility advice.
- CMS policies, moratoria, fee schedules, and enrollment requirements can change. Verify current official guidance.
- National health spending and HRSA totals are context, not DME market size or sales potential.
- Medicare claims datasets can exclude Medicare Advantage and other populations, suppress small counts, and lag the current year.
- A supplier directory entry is not proof of active operations, purchasing intent, or product fit.
- A grant recipient is not automatically a buyer.
- VA schedule coverage does not guarantee an award or order.
- Historical awards reveal patterns but are not open opportunities.

## Next-action checklist

- Define the product category and use case.
- Confirm whether Medicare supplier rules apply to the company and product.
- Verify the current enrollment moratorium status.
- Map relevant VA schedule, FSC, NAICS, and active notice routes.
- Identify funded providers and recipients whose program purpose fits.
- Compare direct, distributor, prime, and partner motions.
- Preserve source dates and data-period limitations.
- Assign a target, contact path, and one validation task.
- Screen out records that rely on broad healthcare keywords only.
- Recheck live official sources before acting.
- Confirm the dataset or award period before comparing organizations.
- Give every monitor item a trigger and recheck date.

## Evidence and source ledger

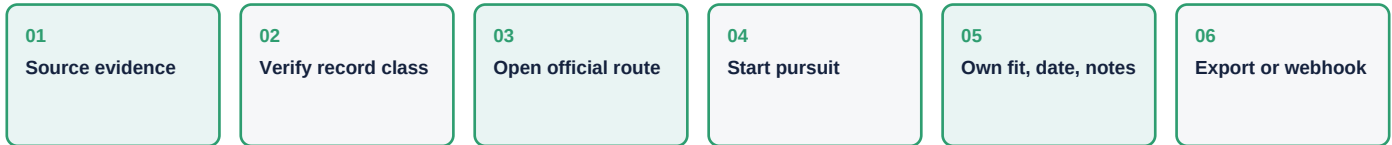
| Claim or use                                                         | Official source                                            | Retrieval date | Caveat                                                                                |
|----------------------------------------------------------------------|------------------------------------------------------------|----------------|---------------------------------------------------------------------------------------|
| Current NHE projections and sponsor context                          | CMS, <a href="#">official source</a>                       | 2026-07-10     | Context only; not DME market size.                                                    |
| DMEPOS enrollment, accreditation, and surety bond steps              | CMS, <a href="#">official source</a>                       | 2026-07-10     | Requirements and exemptions vary; verify with CMS/enrollment contractor.              |
| Temporary nationwide DMEPOS medical-supply enrollment moratorium     | CMS, <a href="#">official source</a>                       | 2026-07-10     | Began 2026-02-27; time-limited and extendable; verify current status.                 |
| DMEPOS fee schedule files                                            | CMS, <a href="#">official source</a>                       | 2026-07-10     | Payment schedule, not demand or buyer intent.                                         |
| DME supplier and service dataset                                     | CMS Data, <a href="#">official source</a>                  | 2026-07-10     | Latest listed data is 2023; Original Medicare FFS scope and suppression apply.        |
| Medical equipment supplier directory                                 | CMS Provider Data Catalog, <a href="#">official source</a> | 2026-07-10     | Directory entries are not opportunity or market-size proof.                           |
| VA Medical Equipment and Supplies Schedule 65 II A                   | VA OPAL, <a href="#">official source</a>                   | 2026-07-10     | Schedule route and categories; inspect current solicitation and SAM.gov notices.      |
| VA schedule ordering context                                         | VA OPAL, <a href="#">official source</a>                   | 2026-07-10     | Ordering rules do not prove a current order.                                          |
| HRSA active and awarded grant datasets                               | HRSA, <a href="#">official source</a>                      | 2026-07-10     | FY2026 is preliminary; active grant definition uses project end date.                 |
| HRSA FY2025 grants dashboard                                         | HRSA, <a href="#">official source</a>                      | 2026-07-10     | Program total, not product demand or company opportunity.                             |
| HRSA health center and site data                                     | HRSA, <a href="#">official source</a>                      | 2026-07-10     | Funded-provider context; validate purchasing role and program fit.                    |
| Round 2028 DMEPOS competitive-bidding categories and target schedule | CMS, <a href="#">official source</a>                       | 2026-07-11     | CMS labels dates as targets; verify registration and bid-window status before acting. |
| Competitive-bidding temporary gap and Round 2028 status              | CMS/CBIC, <a href="#">official source</a>                  | 2026-07-11     | Program-status page, not a contract award or proof that bidding is open.              |
| Competitive-bidding accreditation and licensing readiness            | CMS, <a href="#">official source</a>                       | 2026-07-11     | Requirements are location and product-category sensitive; verify current guidance.    |
| VA FSS product and contract research tool                            | VA OPAL, <a href="#">official source</a>                   | 2026-07-11     | Catalog and contract evidence do not prove current agency demand.                     |

## PRODUCT WORKFLOW

# Turn source evidence into an owned pursuit

Opportunity Scanner first verifies what kind of record it found. Live opportunities retain a current official close date; historical awards become funded-buyer evidence and are never presented as open applications or bids.

## From discovery to a workflow-ready next step




**Opportunity Scanner**  
 OPPORTUNITY SYSTEMS

Product
 Solutions
 Industries
 Resources

Account
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### HISTORICAL AND MARKET CONTEXT

## Funded-buyer evidence and demand signals

These records support buyer, partner, and market research. They are not presented as open applications or bids.

3 evidence rows

Evidence / market signal
 Partner with Recipient
 Priority #1

### NEA grants indicate funded arts organizations and local programming demand

NEA funding supports arts projects and can identify organizations with public funding for performances, programming, education, and cultural access.

USE AS  
 Buyer and demand evidence

**TARGET**  
 Arts nonprofits, cultural organizations, and city arts partners

**CONTACT PATH**  
 Identify funded recipient leadership, programming director, education lead, or partnerships contact.

**NEXT BEST ACTION**  
 Build a recipient target list by discipline, geography, and likely need for performers or teaching artists.

**SOURCE RECORD**  
 National Endowment for the Arts - Grants for Arts Projects  
 Grant / funded recipient signal

**FIRST-TOUCH ANGLE**  
 Reference the public arts objective and offer a curated talent/programming layer to help execute the funded work.

Report view: funded-buyer evidence is separated from live opportunities and carries its source record, target, contact path, and next best action.

### WHY CLASSIFICATION MATTERS

A verified record class determines the action. A live notice may support a direct application or bid; funded-buyer evidence supports buyer, recipient, partner, and market research.

## CUSTOMER WORKSPACE

# Keep the next action connected to the evidence

Open the authoritative route before acting, then turn a qualified finding into work your team can own and advance.

**CUSTOMER WORKFLOW**

### The report becomes a tracked pursuit

Opportunity Scanner opens the authoritative route and organizes the work. It does not submit an application or bid for the customer.

- 1. VERIFY**  
**Open the official source**  
Confirm status, eligibility, timing, and instructions at the authoritative record before acting.
- 2. QUALIFY**  
**Make a fit decision**  
Choose pursue, monitor, partner, or pass and keep the decision attached to the source evidence.
- 3. OWN**  
**Start a pursuit**  
Assign an owner, set an internal next step, and track requirements, documents, notes, and follow-up.
- 4. MOVE**  
**Send qualified context**  
Export the row or send a workflow-ready source, target, contact path, and next action by secure webhook.

**PURSUIT WORKSPACE PREVIEW**

Turn a qualified finding into owned work

| STAGE      | OWNER                     | NEXT STEP                                                                                                   |
|------------|---------------------------|-------------------------------------------------------------------------------------------------------------|
| Qualifying | Business development lead | Build a recipient target list by discipline, geography, and likely need for performers or teaching artists. |

Pursuit view: verify the official source, make a fit decision, assign ownership, and keep the next step attached to the evidence.

**See what a public-sector opportunity scan can reveal in your industry.**

Start with CivicStage to see the full product workflow, then explore industry reports built from fictional companies and public-source examples.

**Scan Your Company Website** **Run a scan**

**Featured product walkthrough**

**ACTIVE PURSUIT**  
**Cultural programming partner path**

Funded-buyer evidence Qualifying

**What stays with the pursuit**

Owner, fit decision, verified due date when one exists, requirements, notes, and the next follow-up.

Qualified context can then be exported or sent through a secure workflow webhook.

Product walkthrough: a fictional company moves from website analysis to sourced findings and a structured pursuit workspace.

**YOUR TEAM STAYS IN CONTROL**

Opportunity Scanner organizes evidence and the pursuit workflow. It does not submit applications or bids on your behalf; your team reviews the official instructions and decides what to send.

## Put this guide to work

Opportunity Scanner reads your company website, finds source-backed public-sector signals, and translates them into target organizations, revenue motions, contact paths, and next actions.

**Run a free company scan at [opportunityscanner.ai](https://opportunityscanner.ai)**

Recheck official sources before any outreach, application, registration, or bid decision.